

**A Toolbox for the Acute Care Nurse Practitioner for Palliative Care
in the Intensive Care Unit**

Cynthia Sturm, MS, RN

The University of Texas Health Sciences Center at Houston,

Cizik School of Nursing

NURS6, Dr. Shannon Warren

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Nurse Practitioners (NPs) were first introduced in the 1960s when there was a severe shortage of access to pediatric primary care. Over time, the use of NPs has increased to wide-ranging roles due to the benefits of using NPs over physicians. Currently, the country is facing a shortage of physicians again, but this time in all specialties (Abdel-Razig & Stoller, 2025). It is this sort of necessity that is driving NPs to take part in critical care services. These NPs have advanced certifications that prepare them for the care of the critically ill, known as an acute care nurse practitioner (ACNP) (Kapu, 2021).

Palliative care is specialized medical care for people with serious illnesses that focuses on relief of symptoms, pain, and stress, with the goal of improving quality of life for both patients and their families. It can be provided at any age and at any stage of a serious illness, and it can be given alongside treatments intended to cure or prolong life. (Sheikh et al., 2022). Hospice care also aims to provide comfort and quality of life, but differs in terms of timing, eligibility, and curative intent. The aim is quality rather than length of life (Sheikh et al., 2022). Additionally, if the patient is in the intensive care unit (ICU), they may be transitioned to “comfort care,” a term used to describe a type of care that is usually enacted after aggressive therapies are no longer working or that the treatment goes against the patient’s wishes. These are measures that provide end of life care which eases any pain, suffering, and allows death with (Tucker et al., 2024).

As time continues and our patient population gets sicker, the need for palliative care increases. Palliative care teams are appearing in hospitals, and they can be consulted for any type of symptom management at any stage of disease, including during curative treatment. In contrast, hospice care has limitations on whom may access it and at what time during the disease process it may be used and cannot be considered while undergoing curative treatment. Many patients are

referred to hospice care directly from the ICU, and hospice services may be initiated there. Each of these services provides great value to the critical care team, but there are limits to their availability, and the ICU medical team may not always have the option to rely on their services. As ACNPs begin playing greater roles in the acute care service areas, they can become part of the essential role in providing palliative care. For the purposes of this paper, the discussion will be limited to the role of ACNPs in ICUs.

ACNPs can make a difference by supporting and providing palliative care in the ICU. Many smaller hospitals lack palliative care options, and even larger hospitals may not have the resources to expend. In these situations, it makes sense for ANCPs to play a larger role in providing palliative care treatment within the ICU. This would ease the burden on the system, as well as provide continuity of care since ACNPs may have more contact with the patients and families than the physician team due factors such as caseloads (Htay & Whitehead, 2021). ACNPs can help identify patients who may benefit from palliative care early in their ICU stay and place consults to initiate supportive interventions (Stilos et al., n.d.).

The question now is how can an ACNP be trained to provide palliative care? One limiting factor is what ACNPs are allowed to do within their scope of practice (per each State Boards of Nursing). However, there are several non-clinical ways to provide aspects of palliative care. ACNPs can speak with, educate, and help guide decision-making of the patients and their families in the end of life. Sharing difficult information with patients and families can be challenging (Teoli et al., 06). A helpful mnemonic device called the SPIKES protocol aids in ACNPs communicating with the patients and their families. **S – Setting** ensures privacy, while minimizing interruptions. **P – Perception** assesses the patients/families understanding of the situation. **I – Intervention** where the ACNP can ask how much the patients/families want to

know about what is going on. **K – Knowledge** is shared clearly and gently, without euphemisms. Finally, **E – Emotions** is defined as the ACNP responding to the patients/families with empathy. This must be combined with non-judgmental and culturally considerate discussion (Teoli et al., 2006). ANCPs can also help patients/families by providing them with knowledge and guidance around goals of care and advanced planning, such as Advanced Directives, Medical Powers of Attorney, and Do Not Resuscitate (DNR/DNI) orders, which are often very confusing and can cause anxiety (Henriksen, 2025).

ACNPs can support patients clinically, within their scope of practice, as well. ACNPs are already trained in the care of the patients' needs, and this can be adjusted to include a palliative "spin." ACNPs have been trained to fully assess their patients, and the palliative care aspect can easily be incorporated. ICU rounding already includes feeding, analgesia, sedation, thromboembolism prophylaxis, head of bed elevation, ulcer prophylaxis, glycemic control, bowel care, indwelling devices, and de-escalation of interventions, remembered by using the mnemonic FAST HUGS BID (Seo et al., 2024). ACNPs assess all patients at least once daily or upon admission, and their assessment can guide their treatment and notify other providers if the patient is experiencing new or worsening symptoms.

In addition, the use of palliative care protocols and order sets can assist ACNPs in providing appropriate and timely interventions (Bell et al., 2023). When ACNPs can use protocols and order sets, there is enhanced autonomy and professional satisfaction, as they are empowered to initiate timely palliative care consults and transitions (Greene et al., 2025). Protocols and order sets created for end-of-life care allow nurse practitioners to:

- Lead or participate in goals-of-care discussions.

- Assist with symptom management, such as pain and dyspnea.
- Transition patients to comfort care or hospice referrals where appropriate.
- Implement standardized clinical protocols for withdrawing or withholding life-sustaining treatments (Bailey et al., 2021).

Order sets help ensure processes such as code status discussions on admission, symptom protocols, and clear steps for withdrawing or withholding treatments are performed consistently across ICU settings (Bailey et al., 2021). Hospitals with established nurse-driven protocols report increased rates of comfort care transitions and hospice referrals, directly impacting patient and family satisfaction (Greene et al., 2025). These order sets should have a clear and concise “triggers” for the ACNP to proceed with initiating certain protocols or referral to specialist palliative care services. These triggers may include poor or incomplete response to medical treatment, persistent or worsening clinical deterioration despite active therapy, and/or presence of difficult-to-manage symptoms such as pain, dyspnea, or anxiety (Hui et al., 2022).

The use of order sets and protocols is essential for increasing ACNP autonomy in the face of palliative care within the ICU. They can be used, in part, to create an ACNP “toolbox” that providers can use to provide appropriate and timely care to patients at the end of life. Other aspects of this toolbox are communication tools such as SPIKE, organized rounding tools such as FAST HUGS BID, and trigger criteria to know when to proceed to referral to specialized palliative care services. This toolbox provides for increased autonomy and fulfillment which may lead to increased patient/family satisfaction, as well as improving quality of care (Collins & Small, 2019; D'Ambruso et al., 2022).

These practices are critical for delivering compassionate, efficient, and standardized care for patients nearing the end of life in the ICU—ensuring their needs and wishes are prioritized while supporting the healthcare team’s role in this challenging context (Kosar et al., 2024). The demand for palliative care practitioners will continue to grow, and the significance and scope of palliative care will keep evolving as the field expands. ACNPs are poised to bridge that gap between palliative care needs and availability of services, leading to improved outcomes and quality of care.

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